Irinotecan · every three weeks

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · irinotecan

Active medicines: Irinotecan

Emetogenic risk: Moderate

Medicine	Dose / route	When
Ondansetron	16 mg PO once	Before Day 1 chemotherapy NCIS V8 moderate-risk row; consider QT risk, low electrolytes, interactions and the individual prescription.
Dexamethasone	8 mg PO once	Before Day 1 chemotherapy NCIS V8 moderate-risk row.
Dexamethasone	8 mg PO daily	Days 2 and 3 after the administered Day 1 chemotherapy NCIS V8 delayed prophylaxis. Reconcile with any other steroid prescription; re-anchor dates if treatment is delayed.
Metoclopramide	10 mg PO up to three times daily as required	Rescue nausea treatment only as prescribed NCIS V8 rescue option; check contraindications, renal function, interactions and prescribed duration.
Atropine sulphate	Per prescribed irinotecan protocol	Before irinotecan when prescribed; assess acute cholinergic symptoms Avoid in glaucoma. This treats/prevents the acute cholinergic syndrome; it is not the delayed-diarrhoea rescue medicine.

Additional phase-specific support

- Antiemetic classification refers to the active treatment day. Reconcile duplicate steroid entries before prescribing.
- No automatic granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) schedule is added; use the patient's neutropenia history, risk and consultant plan.
- Provide a written irinotecan delayed-diarrhoea rescue and escalation plan before treatment; distinguish it from immediate infusion-related cramps/sweating. Fever, severe pain, dehydration or diarrhoea not responding to the prescribed plan needs urgent review.
- The team checks blood counts, kidney/liver function, symptoms, allergies and medicines. Weekly blood counts are part of this irinotecan plan.
- Irinotecan can cause early cramps/sweating during infusion and delayed diarrhoea after 24 hours. You need separate instructions for each. Follow your written rescue plan. Fever, blood, severe pain, reduced urine or inability to drink needs urgent advice.
- Allow for blood tests, review, pharmacy preparation, infusion and observation. A port is not automatically required; ask which access is suitable.
- Use prescribed anti-sickness medicines, pace activities and ask for nutritional, emotional or practical support.
- Ask how and when the team will assess benefit, which blood tests continue during a pause, and what happens after the final dose. Weekly blood counts remain part of the irinotecan plan between visits.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00213 v7 · p3 · NCCP NCIS antiemetics V8 (2025), Table 1 and multi-day principles

Sources checked 2026-11-04 · SACTCheck v0.84.21 · Regimen-level drafts; prescriber and oncology-pharmacy verification required